

A Randomized Trial of Intensive versus Standard Blood-Pressure C

Abstract

Background: The most appropriate target for systolic blood pressure to reduce cardiovascular morbidity and mortality among persons without diabetes remains uncertain.

Methods: We randomly assigned 9361 persons with a systolic blood pressure of 130 mm Hg or higher and an increased cardiovascular risk, but without diabetes, to a systolic blood-pressure target of less than 120 mm Hg (intensive treatment) or a target of less than 140 mm Hg (standard treatment). The primary composite outcome was myocardial infarction, other acute coronary syndromes, stroke, heart failure, or death from cardiovascular causes.

Results: The trial was stopped early after a median follow-up of 3.26 years owing to a significantly lower rate of the primary composite outcome in the intensive-treatment group than in the standard-treatment group (1.65% per year vs. 2.19% per year; hazard ratio, 0.75; $P < 0.001$). All-cause mortality was also significantly lower in the intensive-treatment group (hazard ratio, 0.7

Conclusions: Among patients at high risk for cardiovascular events but without diabetes, targeting systolic blood pressure of less than 120 mm Hg, as compared with less than 140 mm Hg, resulted in lower rates of fatal and nonfatal major cardiovascular events and death from any cause.